

Dietary guidelines for gastrointestinal disorders: key to optimizing practice but more work needed

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 Check for updates

Nutrition and dietary guidelines in gastrointestinal disorders vary in scope, rigour and recommendations, contributing to variations in practice. This Comment identifies nutrition and dietary guidelines for gastrointestinal disorders, namely chronic constipation, irritable bowel syndrome and inflammatory bowel disease, and examines the methodologies used to develop them.

Nutrition and dietary guidelines are essential to ensure patients have access to equitable evidence-based dietary management. In this Comment, we identified nutrition and dietary guidelines via a literature search in MEDLINE (1946 to October 2025; OvidSP) and included those focused exclusively on the nutrition and/or dietary management of chronic constipation, irritable bowel syndrome (IBS) or inflammatory bowel disease (IBD); these conditions were selected because they represent three of the most prevalent gastrointestinal disorders in clinical practice for which nutrition and dietary modification is a central component of management. To ensure that included guidelines reflected national or international consensus, only guidelines endorsed by a learned society or association were considered. Guidelines addressing multiple gastrointestinal conditions were excluded.

Overall, 14 nutrition and dietary guidelines were identified^{1–14}. Ten of the guidelines focused on IBD^{1–3,6–11,14}, with only three on IBS^{4,12,13} and just one on chronic constipation⁵. This predominance is notable given that IBD has a substantially lower population prevalence than either chronic constipation or IBS. Several factors could explain this imbalance in the focus of nutrition and dietary guidelines. Firstly, IBD is an organic condition associated with substantial morbidity and nutritional risk, which might have led professional societies to prioritize guideline development in this area. Secondly, for the same reason, it is possible that robust evidence for nutrition and dietary therapies in IBS and chronic constipation emerged later than evidence for such therapies in IBD, contributing to fewer and more recent guidelines being produced for these conditions. Nevertheless, IBS and chronic constipation are more prevalent disorders that exert considerable effects on quality of life and for which patients commonly desire dietary approaches to their management¹⁵. Furthermore, IBD is commonly managed in secondary care by a specialist multidisciplinary team, whereas IBS and chronic constipation are often managed in primary care by non-specialist doctors and, therefore, evidence-based guidelines

would greatly inform and harmonize practice for non-specialists. Guidelines on IBD are focused on both clinical nutrition and dietary management, whereas those on chronic constipation and IBS focus on dietary management only; this difference is due to high rates of malnutrition in IBD, but also due to exclusive enteral nutrition being a clinical nutrition intervention in Crohn's disease specifically.

Notably, the first nutrition and dietary guidelines were published only 16 years ago in 2010, focused on IBD. Since that time there has been an increasing number of such guidelines published, as well as an increasing number of recommendations contained within each guideline (Fig. 1). Indeed, within the past five years alone, eight guidelines have been published on chronic constipation, IBS and IBD, constituting over half of all nutrition and dietary guidelines available. However, the first and only dietary guidelines in chronic constipation were only published in 2025, clearly addressing a longstanding unmet need. The recent increase in nutrition and dietary guidelines for these gastrointestinal disorders likely reflects the larger volume of evidence available in the past decade, and the greater recognition of the need to align clinical practice with the evidence. Furthermore, there is undoubtedly increased recognition of the crucial part diet plays in the management of gastrointestinal disorders, along with the fact that patients commonly use dietary modifications as a management strategy to control their symptoms, and a desire from them for evidence-based information from health professionals.

There has also been considerable variation in the methodology used in developing nutrition and dietary recommendations. Guidelines vary in how they identified evidence in the literature, appraised the quality of evidence and achieved consensus on guideline statements. Only one (7%) guideline conducted systematic reviews and meta-analyses to underpin all recommendations⁵ and only two (14%) conducted systematic reviews (without meta-analysis) that informed some, but not all statements^{6,13}, relying on systematic searches only for the remainder. Seven (50%) guidelines conducted solely a systematic search of the literature without completing a full systematic review^{1–3,9,11,12,14}, whereas the remaining four (29%) used approaches based on non-systematic literature searches^{4,7,8,10}. Approaches to evidence appraisal were also variable: five (36%) guidelines used the Scottish Intercollegiate Guidelines Network (SIGN) approach^{1,2,6,9,12}, two (14%) used the Grading of Recommendations Assessment, Development, and Evaluation (GRADE) approach^{5,11} and four (29%) used other strategies. Notably, three (21%) guidelines did not appraise the quality of evidence at all^{4,7,8}. Consensus approaches included a Delphi approach in five (36%) guidelines^{1,3,5,6,11}, an unspecified consensus approach in five (36%)^{2,9,10,13}, and four (29%) did not describe any consensus strategy^{4,7,8,12}. The level of agreement to be achieved to retain statements ranged from 75% to 95%. Taken together, these findings highlight

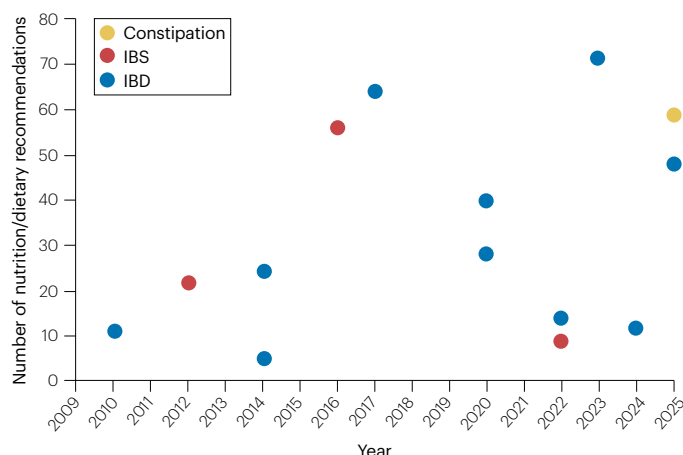


Fig. 1 | Number of recommendations included in nutrition and dietary guidelines of chronic constipation, IBS and IBD over the years. Nutrition and dietary guidelines were searched in MEDLINE (1946 to October 2025; OvidSP), focusing exclusively on the nutrition and/or dietary management of chronic constipation, irritable bowel syndrome (IBS) or inflammatory bowel disease (IBD)^{1–14}.

substantial heterogeneity in the methodological rigour of nutrition and dietary guidelines for gastrointestinal disorders. This inconsistency has important implications for clinical practice, as variability in evidence identification, appraisal and consensus processes might contribute to variable guideline quality, and ultimately to differences in the nature and quality of dietary care that patients receive.

In two cases, guidelines for a specific gastrointestinal disorder were updated by the same society over time. The European Society for Clinical Nutrition and Metabolism (ESPEN) published guidelines for IBD in 2017 (ref. 6), and again in 2020 (ref. 2) and 2023 (ref. 1). All three iterations followed the same approach in identifying and appraising evidence, and used similar consensus approaches, although the required consensus agreement threshold differed between versions. Using similar methodologies, the earliest 2017 iteration included 64 recommendation statements, increasing to 71 by 2023 (refs. 1,6). The British Dietetic Association (BDA) guidelines for IBS were developed and published in 2012 (ref. 12) and again in 2016 (ref. 13). Unlike the ESPEN updates, the BDA iterations followed slightly different methodologies, incorporating more robust methods over time. The 2012 guidelines included 22 evidence statements, increasing to 56 by 2016 (refs. 12,13). Overall, these repeated updates demonstrate that as new research accumulates, guidelines tend to expand in scope and detail, underscoring the necessity of regular methodological review and revision to ensure that nutrition and dietary guidelines remain current, comprehensive and evidence-based.

There is substantial variability in the number and methodological approaches of nutrition and dietary guidelines for chronic constipation, IBS and IBD. Although the volume of guidance has increased over time, the rigour with which evidence is identified, appraised and translated into recommendations remains inconsistent across guidelines and conditions. There is a need for greater standardization, transparency and robustness in guideline development processes to ensure that patients receive reliable, evidence-based dietary care regardless of where or by whom that advice is given. Future guideline

development should include clear reporting of systematic search strategies, systematic reporting and assessment of the identified evidence, and transparent presentation of these methods within guideline methodology sections. Routine use of established appraisal frameworks, such as GRADE or SIGN, together with greater involvement of multidisciplinary stakeholders, as well as patients or the public when relevant, would further strengthen the guidelines. Regular updates, aligned with emerging evidence, will also help to ensure that recommendations remain clinically relevant.

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Competing interests

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